EXECUTIVE ALIGNMENT

From Page Production to Governed Care Experiences

A convergence of Angie King's OOUX discipline and Nalie's Experience Layer for Mayo Clinic's Care Hub.

> This convergence gives Mayo a way to move from page-by-page content production to a governed experience system. OOUX keeps the source truth stable. The Experience Layer makes that truth actionable for patients, caregivers, and referring clinicians in the moments where they are deciding what to do next.

Executive truth alignment for Mayo Clinic's Care Hub evolution.

1. THE OPPORTUNITY

Serious-care patients need decision clarity, not more pages.

CURRENT PRESSURE

Content is accurate but fragmented.

Patients arrive through condition libraries, search, referrals, answer engines, and care pages. Each surface may be useful, but the full decision journey can still feel scattered.

CARE HUB OPPORTUNITY

Make Mayo-specific expertise navigable.

Care Hub can connect general condition understanding with Mayo's differentiated care model without collapsing the boundaries that keep syndicated content clean.

1. **Truth vs. persuasion**
   Patients distrust marketing, but still need evidence and confidence signals before taking action.

2. **Canonical vs. contextual**
   Generic condition truth and Mayo-specific expertise both matter, but they need governed boundaries.

3. **Promise vs. operational reality**
   "We coordinate care for you" must be meaningful without promising physician-level booking or exposing unnecessary system seams.

2. ANGIE'S OOUX LAYER

OOUX defines what exists in the Care Hub system.

SOURCE TRUTH

Canonical care objects and relationships

OOUX separates the real things in the system from pages, views, wrappers, and labels. That matters because every future surface inherits its truth from the same governed object model.

- Care Hub
- Care Pathway
- Condition
- Treatment
- Evidence
- Patient Example
- Support Service
- Care Team
- Department
- Location

WORKING RULE

Objects answer: what exists?

If a label is a view, campaign, answer, page, or presentation wrapper, it should not silently become a new object. The object model stays stable while representations vary.

Decision to keep visible

For the Lung Transplant pilot, Care Hub can be framed as the working decision hub. The final primary-object call should remain explicit until the Mayo team resolves it.

3. NALIE'S EXPERIENCE LAYER

The Experience Layer defines how object truth becomes useful in decision moments.

Entry

FIND

The user is trying to orient: what is this care area, and is it relevant to me?

Likely surfaces: internal search, search engine entry, answer-engine result, topic hub.

Fit and Confidence

TRUST

The user needs to understand whether Mayo has the expertise, care model, and evidence that justify the next step.

Likely surfaces: AI answer, care overview, evidence block, patient example, clinician support.

Eligibility and Action

ACT

The user needs a clear path to request evaluation, understand logistics, and trust that Mayo will coordinate care.

Likely surfaces: Care Hub page, pathway module, request evaluation CTA, support-service content.

The Experience Layer does not compete with OOUX. It gives object truth a patient-facing sequence.

4. THE CONVERGENCE

Design Concepts are the bridge between OOUX and the Experience Layer.

OOUX DESIGN

Canonical objects

Care Hub, pathway, evidence, team, support, conditions, patient examples, locations.

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DESIGN CONCEPTS

Reusable care ideas

Example: "Lung Transplant Care as a decision-support experience."

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EXPERIENCE LAYER

Rendered moments

Search entry, AI answer, Care Hub page, request-evaluation path.

What stays stable

Object definitions, relationships, claims, proof points, CTA ownership, governance rules.

What can vary

Surface, sequence, emphasis, copy depth, visual module, and next-step framing.

What this prevents

Duplicate pages, competing labels, ungoverned AI answers, and one-off experience decisions.

What this enables

Reusable patterns that can scale from one care family to many without losing Mayo-specific nuance.

5. EXAMPLE

Lung Transplant Care Hub rendered across three experiences.

EXPERIENCE	WHAT IT ASSEMBLES	PATIENT-FACING PURPOSE
Search entry Find the right Mayo care area.	Care Hub summary, who we treat, related pathway, support-service labels.	Help the patient recognize that Mayo has a relevant lung transplant care path.
AI answer experience Answer the confidence question.	Value proposition, evidence bundle, care-pathway summary, support-service explanation.	Explain why Mayo Clinic may be worth considering for advanced lung disease care.
Care Hub page Guide the action moment.	Who we treat, how care works, outcomes and proof, care team, travel and support, patient examples.	Move from understanding to request evaluation without overpromising physician-level booking.

> One object model, multiple rendered experiences, one governed decision journey.

6. WHAT CHANGES FROM THE CURRENT PDF

Keep the insight. Upgrade the artifact.

WHAT WORKS

The underlying model is strong.

The OOUX and Experience Layer relationship is correct: objects are source truth, experience patterns are orchestration logic, rendered experiences are instances.

The Lung Transplant example is the right kind of concrete demonstration.

WHAT MUST CHANGE

The current artifact reads like a web export.

It needs executive sequencing, no internal links, no placeholder organization names, no repo references, no broken page flow, and no unexplained terminology.

The result should feel like truth alignment that can stand on its own, not a raw browser page printed to PDF.

1. **Replace placeholders**
   Use Mayo Clinic language throughout.
2. **Remove internal surfaces**
   No package links, repository paths, substrate references, or implementation-only labels.
3. **Clarify roles**
   Angie owns OOUX discipline; Nalie contributes the Experience Layer terminology and design lens.
4. **Control pagination**
   Each page should be intentionally composed, with no card split across pages.

7. WHY IT MATTERS

A governed experience system is scalable, AI-ready, and easier for engineering to trust.

Better patient guidance

Decision moments become clearer across search, AI answers, and Mayo-owned pages.

Governed content reuse

Core claims, evidence, and CTAs come from canonical objects rather than one-off page copy.

AI readiness

Answer engines can draw from structured, attributed content without blurring the syndicated vs. Mayo-specific boundary.

Engineering alignment

Objects, patterns, assembly rules, and rendered instances can become contracts rather than subjective design notes.

RECOMMENDED NEXT MOVE

Create the Lung Transplant pilot matrix.

For each decision moment, define the primary object, supporting objects, required attributes, proof points, CTA, allowed surfaces, and validation rules.

DECISION TO RESOLVE

Primary object stance

Decide whether Care Hub is the resolved primary object, a pilot recommendation, or a working assumption for this first Care Hub example.